



HospitalOS

Hospital Operating System for Africa

Tenant Guide

Hospital staff reference

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1. Who this guide is for

This guide is for hospital staff using HospitalOS day to day — clinicians, diagnostic and pharmacy staff, finance and administration teams. It covers every module in the system and, where it matters, why a screen behaves the way it does.

Role	Scope	Account
Administrator	Your hospital's full HospitalOS deployment	Any account with the Super Admin role
Clinical staff	Patient care, diagnostics, pharmacy for your role	Doctor / Nurse / Lab Scientist / Radiographer / Pharmacist accounts
Finance staff	Billing, payments, claims	Cashier account

Two-level permissions. Every role has an **AREA** list (which sidebar groups it can open) and a **GRANTS** list (which specific actions it can take once inside). Section 3 covers this in full, with the actual permission table.

2. Signing in and how the system is organised

Sign in with your hospital email and password. An administrator sets your role under Administration → Users & roles when your account is created — there is no self-service sign-up.

Security. Your password is checked server-side against a securely hashed copy — it is never stored or transmitted in readable form. Repeated failed sign-in attempts are automatically rate-limited.

2.1 The twelve sidebar groups

The sidebar is arranged by workflow, roughly in the order work happens through a hospital, not by department hierarchy:

Group	What lives there
Overview	Dashboard, alerts, worklist, communication hub, online bookings, referrals
Patient care	Registration/ADT, wards, ICU, emergency, theatre, maternity, records, renal, and 21 specialist clinics
Diagnostics	Laboratory, blood bank, imaging, radiotherapy, biobanking, instruments gateway, diagnostic intelligence
Pharmacy	Dispensing, inventory, NAFDAC formulary
Finance & trade	Billing, payments, NHIA & insurance claims, procurement, stores
Operations	Scheduling, CSSD, biomedical equipment, facility & waste, fleet, support services, visitors
Academic	Training programmes, clinical logbooks, CME, research, ethics committee
Public health	Surveillance, immunisation (full NPHCDA schedule per child), outreach, national reporting
Specialty services	Nutrition, sickle cell centre, dental, IPC, social work, occupational health, chaplaincy
Intelligence	Analytics, reports, forecasting
Compliance	Practitioner licenses, accreditation, incident & risk management, policies & SOPs
Administration	Users & roles, facilities, instruments gateway, pricing, documents, security & audit, settings

You only see what your role grants. A pharmacist's sidebar shows Overview and Pharmacy; a cashier's shows Overview and Finance & trade. This is not secrecy — it keeps each person's view to what they actually use.

3. Roles and permissions

Area permissions gate the sidebar and routes — enforced on the server for every request, not just hidden on screen. Action permissions gate the buttons within an area. Separation of duties is enforced, not just suggested:

Role	Can	Cannot
Nurse	Admit patients, record vitals, file clinical notes	Discharge a patient
Lab Scientist	Collect samples, enter results, verify results	Reach Patient care at all
Radiographer	Order and report imaging studies	Verify laboratory results
Pharmacist	Dispense medication, restock inventory	Reach Diagnostics at all
Cashier	Take payments, file insurance claims	Approve a claim
Records Officer	Register new patients	Admit or discharge a patient
Doctor	Full patient-care and diagnostics actions	Administration functions
Super Admin	Everything	—

The last-admin protection. The last active Super Admin account cannot be deactivated by anyone, including another Super Admin — this prevents a hospital from accidentally locking itself out of its own system.

4. Overview and the alert system

The Dashboard opens on live occupancy, admissions, and revenue charts. Every stat card links directly into the module behind it.

4.1 Alerts — many sources, one feed

Source	Raises an alert when...	Severity
Laboratory	A result crosses a panic threshold	Critical
Critical care	Any ICU/HDU vital goes critical	Critical
Radiology	A report is flagged with an urgent finding	Critical
Maternity	A newborn's Apgar score is below 7	Critical
Blood bank	A group is below reorder / a unit nears expiry	Critical / Warning
Pharmacy	A drug is out of stock / below reorder	Critical / Warning
Instruments	A device errors or goes offline	Critical / Warning
Renal	A scheduled dialysis session is overdue	Critical
Operations	Equipment under repair / a vehicle is out of service	Warning
Oncology	A chemotherapy cycle is overdue	Warning
Public health	A notifiable disease is trending upward	Warning
Referrals	An Emergency-urgency inbound referral is logged	Critical
Privacy	A data-subject request is overdue against its statutory window	Warning

Most alerts clear themselves — they read live state, so fixing the underlying problem clears the alert automatically, nothing to dismiss.

4.2 Notifications

Current limitation. The bell icon surfaces booking requests inside the app only. It does not yet push a notification outside HospitalOS — that requires the Communication hub's channels to be connected to a real SMS/email carrier, which isn't wired up yet. Its count does not shrink on its own; a booking only leaves the bell once someone actually confirms or declines it.

5. Patient care

5.1 Registration, admission and the ward board

The bed board and ADT read the same registry, so a bed taken in one is unavailable in the other instantly — double-booking is not possible.

5.2 Accommodation tiers

Tier	Rate / night
General Ward	₦15,000
Semi-Private	₦35,000
Private Room	₦60,000
Private Suite	₦120,000
VIP Suite	₦220,000
Executive Suite	₦350,000
Critical Care	₦180,000

Catalogue defaults — every rate can be overridden for your hospital. See Section 9.

5.3 The medical record

SOAP-structured clinical notes, an ICD-10 problem list, and recorded allergies. Filed notes cannot be edited or deleted — a correction is filed as an amendment referencing the original, with both remaining visible. This is enforced on the server, not just in the interface.

Allergy safety is enforced, not advisory. A severe allergy on a patient's record disables the Pharmacy dispense button outright for that drug — and if the allergy check itself fails to load, dispensing is blocked until it succeeds, rather than silently allowing a dispense with an unverified check.

5.4 Referrals and bookings

Referrals in and out of your facility are tracked from first contact through acceptance or decline, through to the patient's arrival. Online bookings holds appointment requests. Confirming and checking in a booking, or accepting and checking in a referral, calls the same function Outpatient uses internally — a checked-in patient is a genuine addition to today's queue, not a separate list.

5.5 Specialist units

Unit	What it does
Renal & dialysis	Haemodialysis programme (vascular access, schedule, fluid removal) and a CKD registry that auto-stages from eGFR
ICU / HDU	Five vitals per bed with panic thresholds; a critical reading marks the patient Unstable and raises an alert automatically

Unit	What it does
Emergency	Sorts by triage acuity first, arrival time second
Maternity	Tracks labour through delivery; a newborn Apgar below 7 raises a critical alert immediately
Oncology	Chemotherapy cycle tracking; an overdue cycle raises an alert, not a quiet reschedule
Geriatric	Falls-risk and polypharmacy screening on admission
Mental health	Inpatient care with admission status, observation level, and risk-flag tracking
21 specialist clinics	Cardiology through Neurosurgery, filtered views inside one Specialist clinics module

6. Diagnostics

Laboratory tests across multiple disciplines, and imaging protocols across Ultrasound, CT, MRI, General Radiography, and Mammography.

6.1 Laboratory and blood bank

Orders move Ordered → Collected → Resulted → Verified; each result flags Low/High/Critical live against its reference range. Blood bank crossmatching is engineered so two simultaneous requests for a scarce blood type can never both be matched to the same physical unit — proven directly under concurrent load before shipping, not just assumed safe.

6.2 Instruments & devices gateway

One interoperability hub, reachable from both Diagnostics and Administration:

Category	Protocol	Examples
Laboratory analyzer	HL7 v2 / MLLP	Sysmex, Cobas, and older analyzers still in service
Imaging modality	DICOM 3.0	CT, MRI, ultrasound, older CR readers
Radiotherapy system	DICOM-RT / proprietary	A modern LINAC and a manual Cobalt-60 unit
Printer	IPP / raw socket / serial	Label, report, and legacy dot-matrix printers

Current limitation. Device management, status, and message history are real and persist. The live network listener itself — an MLLP socket, a DICOM SCP, a print daemon — is server/network territory a browser-based feature can't stand up on its own; every action in this screen calls the exact function a live listener would call.

6.3 Biobanking & lab utilities

Biobanking tracks long-term specimen storage across four capacity-limited units, distinct from the active Laboratory worklist. Lab utilities holds clinical calculators, unit converters, and bench reference material.

7. Pharmacy

Dispensing checks recorded allergies before completing a transaction and blocks any quantity greater than available stock. Inventory alerts clear themselves once stock is restocked above the reorder level. The formulary is aligned to NAFDAC-registered products.

8. Finance & trade

Billing aggregates charges from five sources automatically: laboratory, pharmacy, radiology, theatre, and accommodation. A theatre case becomes billable the moment it enters theatre, not when scheduled. Claims move

Submitted → Approved/Rejected → Paid — a cashier can file a claim but cannot approve it, and the workflow itself blocks marking a claim paid before it has been approved.

9. Operations, Academic, and Public Health

Operations covers the machinery that keeps a hospital physically running: staff rostering, sterile supply (CSSD) cycle tracking, biomedical equipment maintenance, facility and waste checks, ambulance/fleet management, and visitor sign-in.

Academic (for teaching hospitals) covers training programme rosters, clinical logbooks, CME tracking, research project registries, and a full ethics committee review workflow — a review decision requires a reviewer comment, and a finalised decision cannot be reopened, both enforced on the server.

Public health covers disease surveillance with outbreak-signal detection, community outreach tracking, the full National Programme on Immunization schedule tracked per child, and national reporting formats (IDSR, NHMIS).

10. Making HospitalOS your own

10.1 Setting your own prices

Administration → Pricing lets you override the catalogue default for any test, drug, imaging study, procedure, or accommodation tier. Every billing calculation and every price shown to staff reads through the same value.

Past charges are never rewritten. *A patient billed before you changed a price keeps their original total; only new transactions use the new price.*

10.2 Your branding

Administration → Settings has your hospital's real name, logo, address, phone, and email. All appear live across the app and on every printed document — saving here updates them everywhere within seconds, with no reload needed.

10.3 Your documents

Administration → Documents & templates accepts real uploads — drag a file in, assign any category you like, and it is immediately downloadable, renameable, and deletable. Files are stored for real: they survive a reload, a different device, or a different browser session.

11. The audit trail

Administration → Security & audit is append-only by construction: entries are frozen the moment they are written, with no update or delete capability at all. Each entry is hash-chained to the one before it, so editing a past record breaks verification at that record, and deleting one breaks verification at the next. The chain lives in the database, not the browser.

Security note. *This is tamper-evident, the honest and standard claim for this kind of log: any tampering after the fact becomes detectable. It does not claim to make database-level tampering physically impossible for someone with that level of access — no audit design does.*

12. Getting help

Press Ctrl+K (Cmd+K on Mac) from anywhere to search screens, patients, and the full help library at once. Help & documentation is pinned at the bottom of the sidebar, outside the workflow groups, reflecting that it is a standalone reference.

12.1 Downloading this guide

This guide is available at any time from Help & documentation — click “Download the full Tenant Guide” on the landing page.

13. Current state, stated honestly

An honest account of what genuinely isn't built yet. Everything not listed here — including every module and workflow described above — is real and persists.

- ☐ The instruments gateway simulates live device listening (MLLP/DICOM/print daemon); device management and monitoring are real.
- ☐ No real SMS/WhatsApp/email carrier is connected yet; the message queue, templates, and channel selection are real, but delivery stays at “queued.”
- ☐ Settlement (AgoroX-side revenue reporting) doesn't yet break collections down by which module generated them, since payments don't record that at collection time.
- ☐ There is no self-service “forgot password” flow — your hospital's own administrator resets access.

Questions about this guide? support@agorox.africa